

D400

End-of-Life Care

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 62-year-old woman with advanced heart failure is still getting treatment to improve her heart function, but she has severe shortness of breath. Her cardiologist trained at a top program and she has two dogs. Which service can help her now while she continues treatment?

- A. No service until treatment ends.
- B. Hospice
- C. Hospice, since heart failure is always terminal.
- D. Palliative care

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A man with cancer has a prognosis of about three months and has decided to stop chemotherapy to focus on comfort at home. Which service fits?

- A. Curative chemotherapy with palliative care.
- B. Acute hospital admission.
- C. Palliative care only, because hospice is only for the last week.
- D. Hospice

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A retired bus driver with ALS wants his daughter to make medical decisions if he can no longer speak. He enjoys baseball and has lived in the same town for 40 years. Which document does this?

- A. A last will and testament.
- B. A living will.
- C. A durable healthcare power of attorney naming his daughter.
- D. A POLST only.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Paramedics are called to the home of a seriously ill woman in cardiac arrest. Which document gives them a medical order they can follow on the spot?

- A.** Her daughter's verbal statement of her wishes.
- B.** A signed POLST form.
- C.** A healthcare power of attorney.
- D.** A living will in her desk drawer.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

Two months after his wife's death, a man says one day he feels at peace and the next day is angry at her doctors. He has returned to his job as an electrician. How should a care worker interpret this?

- A.** He is stuck and failing to move to acceptance.
- B.** He is fine since he is back at work.
- C.** He needs a psychiatric referral for any anger.
- D.** It is a normal, non-linear grief pattern.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A daughter feels deep sadness and starts to mourn while her mother is still alive in hospice. What is this called?

- A.** Anticipatory grief
- B.** Complicated grief
- C.** Denial
- D.** Acceptance

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A family asks the team not to tell their dying father his diagnosis, saying in their tradition the family carries bad news. The father is fluent in English and was a university professor. What is the best approach?

- A. Tell him everything because patients always have a right to full disclosure.
- B. Tell him, since a professor would want facts.
- C. Ask the father how much he wants to know and who he wants to receive information.
- D. Follow the family's request without asking him.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A father in the ICU cannot speak. His three adult children disagree about whether to continue a ventilator. The ICU recently won a quality award. How should the team open the family meeting?

- A. Follow the wishes of the oldest child.
- B. Share the ICU's award to build confidence.
- C. Ask what the family understands and what their father valued and would want.
- D. Present every treatment option and ask them to vote.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A woman in the last days of life has stopped eating, her breathing is irregular, and there is a rattling sound when she breathes. Her son, who flew in from another state, is upset. The room faces a garden. What should the care team do?

- A. Tell the son these signs always mean death within an hour.
- B. Explain that these changes are expected, and provide mouth care and repositioning for comfort.
- C. Insert a feeding tube so she does not starve.
- D. Call 911 for the breathing sounds.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A wife says, "I'm scared I won't know what to do when he dies at home." She has been a nurse's aide for 20 years. Which response shows empathy?

- A. "It sounds frightening to face this at home. Let's talk about what worries you most."
- B. "You've been an aide for 20 years, so you'll know what to do."
- C. "I feel so sorry for you."
- D. "Don't worry, everything will be fine."

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

Which statement is sympathy rather than empathy?

- A. "You seem angry about how this happened."
- B. "Oh, you poor thing, I feel so bad for you."
- C. "This seems really hard for you."
- D. "Tell me more about what you are feeling."

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A hospice aide who has cared for eight dying patients this month says she feels numb and dreads coming to work. She drives a new car and is taking an online class. What is the best response from her supervisor?

- A. Recognize possible compassion fatigue and connect her with peer support, debriefing, and self-care time.
- B. Give her more patients so she stays busy.
- C. Suggest she drop the online class.
- D. Tell her to push through because strong caregivers don't need help.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 62-year-old woman with advanced heart failure is still getting treatment to improve her heart function, but she has severe shortness of breath. ~~Her cardiologist trained at a top program and she has two dogs.~~ Which service can help her now while she continues treatment?

Target word: right service now **Decides:** She is continuing curative treatment.

A	Inverted operation	Palliative care is available at any stage.
B	Confusion set	Hospice requires stopping curative treatment and a limited prognosis.
C	Absolute language	Not always; her prognosis and choices decide eligibility.
D	Correct	Palliative care relieves symptoms alongside ongoing treatment.

2. Answer D Concept 1 • transfer

A man with cancer has a prognosis of about three months and has decided to stop chemotherapy to focus on comfort at home. Which service fits?

Target word: reverse: from choice to service **Decides:** Limited prognosis and stopped curative care.

A	Inverted operation	He chose to stop chemotherapy.
B	Adjacent category	Not needed for comfort goals at home.
C	Wrong stem number	Hospice covers roughly the last six months, not one week.
D	Correct	He meets both hospice conditions.

3. Answer C Concept 2

A retired bus driver with ALS wants his daughter to make medical decisions if he can no longer speak. ~~He enjoys baseball and has lived in the same town for 40 years.~~ Which document does this?

Target word: which tool names a decision maker **Decides:** Naming an agent.

A	Adjacent category	Handles property after death.
B	Confusion set	States treatment wishes but does not name a decision maker.
C	Correct	It names an agent to decide when he cannot.
D	Dropped element	A medical order about treatments, not a named agent.

4. Answer B Concept 2 • transfer

Paramedics are called to the home of a seriously ill woman in cardiac arrest. Which document gives them a medical order they can follow on the spot?

Target word: reverse: which tool is an order **Decides:** EMS follows medical orders.

A	Adjacent category	Not an order; EMS follows written orders.
B	Correct	A portable medical order recognized across settings.
C	Inverted operation	Names an agent; it does not itself give an order.
D	Confusion set	States wishes; not a medical order for EMS.

5. Answer D Concept 3

Two months after his wife's death, a man says one day he feels at peace and the next day is angry at her doctors. ~~He has returned to his job as an electrician.~~ How should a care worker interpret this?

Target word: grief pattern **Decides:** Back-and-forth reactions are normal.

A	Inverted operation	Assumes a fixed order.
B	Camouflage	Returning to work does not measure grief.
C	Absolute language	Anger is a common grief reaction.
D	Correct	Stages are not a fixed sequence.

6. Answer A Concept 3 · transfer

A daughter feels deep sadness and starts to mourn while her mother is still alive in hospice. What is this called?

Target word: reverse: name the concept **Decides:** Grief before the death.

A	Correct	Grief that begins before a loss.
B	Confusion set	Prolonged, disabling grief after a death.
C	Adjacent category	A reaction of refusing to accept the facts.
D	Inverted operation	Peace with the loss, not mourning ahead of it.

7. Answer C Concept 4

A family asks the team not to tell their dying father his diagnosis, saying in their tradition the family carries bad news. The father is fluent in English and ~~was a university professor.~~ What is the best approach?

Target word: handling a cultural request **Decides:** Ask the patient about his own disclosure preference.

A	Absolute language	Patients can choose to defer information to family.
B	Camouflage	Assumes from his job instead of asking.
C	Correct	Respects both culture and the patient's own wishes.
D	Dropped element	Leaves out the patient's own preference.

8. Answer C Concept 5

A father in the ICU cannot speak. His three adult children disagree about whether to continue a ventilator. ~~The ICU recently won a quality award.~~ How should the team open the family meeting?

Target word: first step in goals-of-care meeting **Decides:** Start with the patient's values.

A	Adjacent category	Not how surrogate decisions are made without an agent.
B	Camouflage	Irrelevant to the goal.
C	Correct	Anchors the goal in the patient's wishes.
D	Inverted operation	Starts with options before values; voting is not a care goal.

9. Answer B Concept 6

A woman in the last days of life has stopped eating, her breathing is irregular, and there is a rattling sound when she breathes. Her son, ~~who flew in from another state~~, is upset. ~~The room faces a garden~~. What should the care team do?

Target word: comfort response **Decides:** Expected signs of dying.

A	Absolute language	Timing varies; absolute claims mislead.
B	Correct	Comfort measures and family teaching.
C	Inverted operation	Forcing nutrition adds burden without benefit at this stage.
D	Adjacent category	Expected secretions are not an emergency.

10. Answer A Concept 7

A wife says, "I'm scared I won't know what to do when he dies at home." ~~She has been a nurse's aide for 20 years~~. Which response shows empathy?

Target word: empathic response **Decides:** Acknowledge the feeling.

A	Correct	Names her feeling and invites her to share.
B	Camouflage	Uses her job to dismiss her feelings.
C	Confusion set	Sympathy — pity from outside rather than understanding.
D	Absolute language	False reassurance dismisses the fear.

11. Answer B Concept 7 · transfer

Which statement is sympathy rather than empathy?

Target word: reverse: identify sympathy **Decides:** Pity from the outside.

A	Adjacent category	Reflecting emotion is empathy.
B	Correct	Feeling sorry for someone is sympathy.
C	Inverted operation	Naming the feeling from their view is empathy.
D	Confusion set	An empathic invitation.

12. Answer A Concept 8

A hospice aide who has cared for eight dying patients this month says she feels numb and dreads coming to work. ~~She drives a new car and is taking an online class~~. What is the best response from her supervisor?

Target word: supporting the caregiver **Decides:** Signs of compassion fatigue.

A	Correct	Protects her ability to keep caring.
B	Inverted operation	Increases the exposure causing the problem.
C	Camouflage	Treats a side detail as the cause.
D	Absolute language	Denies a real occupational risk.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	2	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	6	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Palliative care vs. hospice	Palliative care relieves symptoms at any stage and can run alongside curative treatment; hospice is palliative care for people with a prognosis of about six months or less who have chosen to stop curative treatment.	Believing palliative care means giving up, or that hospice allows ongoing curative treatment under the Medicare benefit.	Prognosis and whether curative treatment continues.
2	Advance directive tools	A living will states treatment wishes; a healthcare power of attorney names an agent to decide when the person cannot; a POLST is a signed medical order for seriously ill patients that travels across settings.	Treating a living will as a medical order, or thinking the agent decides while the patient can still speak for herself.	Which document names a person, states wishes, or is an order EMS can follow.
3	Grief is not a straight line	Kübler-Ross stages are common reactions, not a fixed sequence; anticipatory grief can start before death, and grief that stays intense and disabling for a long time may need referral.	Expecting people to move through stages in order, or to “finish” grieving on schedule.	A family member moving back and forth between reactions.
4	Culture shapes dying and death	Ask each person and family about their beliefs, rituals, and disclosure preferences rather than assuming from ethnicity or religion; honor rituals within safety limits.	Applying a cultural stereotype or a single “right” way to die.	A family request about disclosure, body care, or rituals.
5	Reaching a common care goal	In a family meeting, the team first asks what the patient values and understands, gives clear information, and then helps the family agree on a goal that reflects the patient’s wishes.	Opening with a list of treatment options, or letting the loudest family member decide.	A family split over what to do next.
6	Comfort at the end of life	Near death, changes like decreased eating, mottled skin, irregular breathing, and noisy secretions are expected; care focuses on comfort — mouth care, repositioning, and symptom relief — not forcing food or fluids.	Pushing food and fluids or treating expected signs as emergencies.	Family distress over not eating or noisy breathing.

#	Concept	Rule	Trap	Tell
7	Empathy vs. sympathy	Empathy means understanding and naming the other person's feelings from their view ("This is so hard"); sympathy is feeling sorry for them from outside, and false reassurance shuts feelings down.	Offering reassurance or advice ("Everything will be fine") instead of acknowledging the emotion.	A family member expresses fear, anger, or sadness.
8	Compassion fatigue	Repeated exposure to suffering can cause compassion fatigue; self-care, peer support, and debriefing protect the caregiver's ability to stay present.	Believing strong caregivers should push through without support.	A staff member who feels numb, irritable, or dreads work after many deaths.

Course scope (public description): End-of-Life Care focuses on the Connected Care model as it applies to the final stage of life. This course will explore ethnic and cultural factors that affect an individual's response to death and dying. This course will cover planning and implementing ideal interventions to help individuals, families, and groups cope and agree on a common care goal. This course will also discuss empathy and compassion in healthcare. There are no prerequisites for this course.